

Healthcare Data Governance Case Study

Document Information

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Executive Summary

This case study illustrates how a representative multi-facility healthcare provider implemented data governance to improve patient data quality, strengthen HIPAA compliance posture, and support clinical analytics initiatives. It is intended as an illustrative reference — adapt the specifics to your own organization's context.

Organization Profile: Regional healthcare system, 6 facilities, ~8,000 employees, EHR plus multiple departmental clinical systems.

1. Business Context

1.1 The Challenge

- Patient records were duplicated across EHR, billing, and departmental systems, creating risk of misidentification
- HIPAA compliance relied heavily on manual access reviews with no automated classification of PHI
- Clinical analytics and population health initiatives stalled due to inconsistent data definitions across facilities
- No formal process existed to track data lineage from clinical source systems into reporting and research datasets

1.2 Regulatory Drivers

- HIPAA Privacy and Security Rules (PHI classification, access controls, breach notification)
- State health information privacy laws
- Value-based care reporting requirements to payers and CMS

2. Governance Approach

2.1 Program Structure

A Data Governance Council was established with the Chief Medical Information Officer and Chief Privacy Officer as co-sponsors, ensuring both clinical and compliance perspectives were represented from the outset.

2.2 Phased Rollout

1. **Foundation (Months 1-4):** Charter approval; classification scheme extended to explicitly define PHI sub-categories
2. **Pilot (Months 5-8):** Patient Master Data domain selected as pilot, addressing duplicate record risk across facilities
3. **Expansion (Months 9-15):** Extended to clinical quality reporting and population health analytics domains
4. **Operationalization (Months 16-20):** Governance metrics embedded into compliance committee and quality committee reporting

2.3 Key Capabilities Implemented

- Patient identity resolution rules layered on top of the data quality engine to flag likely duplicate records
- PHI classification applied at the data element level (not just system level) across the catalog
- Automated lineage from EHR and departmental systems through to clinical quality and regulatory reports
- Role-based Data Steward model spanning clinical informatics, compliance, and IT

3. Results

Metric	Before	After (20 Months)
Estimated duplicate patient record rate	~8%	<1.5%
PHI-classified data elements documented	0%	89%
Time to assemble a population health dataset	4-6 weeks	1 week
HIPAA access review cycle time	6 weeks	2 weeks

4. What Worked Well

- Co-sponsorship from both clinical and privacy leadership prevented the program from being perceived as "just an IT/compliance project"
- Defining PHI classification at the data element (not just dataset) level gave much more precise access control and reporting
- Tying the patient identity resolution pilot to a widely felt clinical pain point (duplicate charts) created strong early buy-in

5. Challenges Encountered

- Departmental "shadow systems" (spreadsheets and local databases maintained by individual clinics) were harder to catalog and bring into governance scope than core enterprise systems
 - Clinical staff had limited bandwidth for steward responsibilities during the pilot, requiring lighter-weight steward role definitions for clinical domains
 - Reconciling differing data definitions across facilities (e.g., what counts as a "readmission") took longer than planned
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6. Lessons for Other Healthcare Organizations

- Classify PHI at the data element level from the start — retrofitting this later is significantly more effort
 - Design steward roles for clinical staff to require minimal time investment; pair them with a dedicated governance analyst who can do the detailed work
 - Prioritize identity resolution early — duplicate patient records undermine trust in every downstream initiative
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Document Control:

- This is an illustrative template case study for adaptation to your organization
- Replace illustrative metrics and details with your own organization's data before external use
- Pair with the HIPAA Data Handling template in Section 07